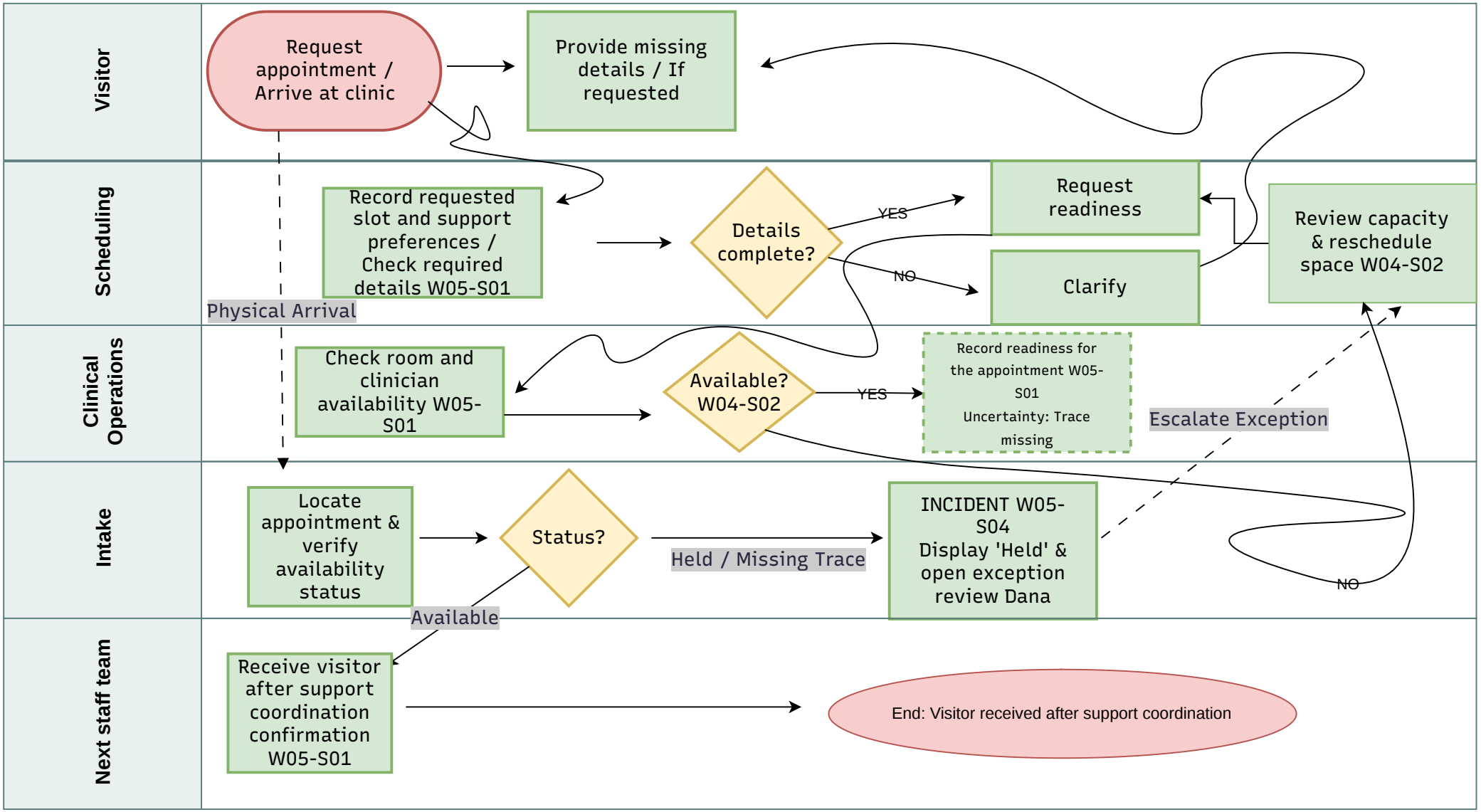
# CIT 140 · Week 5 | Current Appointment Process

Replace this line with your process start and end. Add supported steps and decisions in the responsible role's lane. Label decision branches and source IDs.




Symbol key (copy these shapes into your lanes; they are not case steps)



**Mapping the current workflow revealed that booking confirmations and clinical readiness decisions are separate records. This allows an appointment to appear confirmed to a visitor while Intake still sees it as held [W05-S01, W05-S03]. The handoff to Intake is unclear; when a status is held, Dana opens an exception review with Scheduling and Clinical Operations, but no source specifies who provides the clarification or the expected turnaround time [W05-S03]. Additionally, the incident timeline lacks a completed readiness decision prior to the visitor's arrival, though this absence does not prove it never occurred [W05-S04]. While Week 4 defined clinic needs via testable requirements, this week mapped roles, sequencing, and bottlenecks—revealing a process gap that the standalone requirements list missed.**

# Week 5 | Scope and Findings

**1 · Process scope**
Your name: Jose Becerra. Process purpose: To map and analyze the current clinic workflow to identify task sequencing, responsible roles, and process bottlenecks in appointment management. Start: Visitor requests or submits an appointment booking. End: Visitor arrives at the clinic and the clinical readiness decision is addressed. Included roles: Visitor, Intake, Dana, Scheduling, and Clinical Operations. What falls outside your diagram: Post-intake medical treatment, long-term patient follow-up, and external billing or insurance processes.

**2 · Check the two paths**
Trace an ordinary appointment: For a standard appointment, the visitor books a slot, the system generates a confirmation, and the clinical readiness decision is successfully completed before they arrive. Trace the incident appointment: For the incident appointment, the system showed the booking as confirmed to the visitor, but internally the status remained "held" within Intake, triggering an exception review by Dana. Explain the gap supported by W05-S04: The incident trace shows that the visitor arrived at the clinic without any completed or documented clinical readiness decision on file. State what the trace cannot prove: The absence of the record in the timeline does not definitively prove the readiness decision never happened; it only proves it was not recorded in the system [W05-S04].

3 · Evidence and model review
Source IDs beside steps: Booking confirmation vs. Held status in Intake: `[W05-S01, W05-S03].` Dana opening an exception review: `[W05-S03].` Absence of a readiness decision in the trace: `[W05-S04].` Correction made after checking: Corrected the process model to split "booking confirmation" and "clinical readiness decision" into two distinct data records, as the initial model incorrectly assumed they were a single tied process. Remaining uncertainty: It remains unknown which specific role or individual is responsible for producing the clarification when an appointment is held, as well as the expected turnaround time for this action [W05-S03]. Person or record needed to resolve it: Consultation with Scheduling and Clinical Operations leads, or a review of internal standard operating procedures (SOPs), is required to formally define this role and timeline.

**4 · Learning reflection (reuse your Discord paragraph)**

Mapping the current workflow revealed that booking confirmations and clinical readiness decisions are separate records. This allows an appointment to appear confirmed to a visitor while Intake still sees it as held [W05-S01, W05-S03]. The handoff to Intake is unclear; when a status is held, Dana opens an exception review with Scheduling and Clinical Operations, but no source specifies who provides the clarification or the expected turnaround time [W05-S03]. Additionally, the incident timeline lacks a completed readiness decision prior to the visitor's arrival, though this absence does not prove it never occurred [W05-S04]. While Week 4 defined clinic needs via testable requirements, this week mapped roles, sequencing, and bottlenecks—revealing a process gap that the standalone requirements list missed.

**5 · Portfolio evidence**
[Editable model filename and Week05/Evidence/Week05_Discord_Post.png or .jpg. If Discord is unavailable, identify the equivalent reflection used here.]
Save the .drawio file; export both pages as one PDF; check readability; commit and push.